2026-11-02

Business Plan: A Hawaiʻi Retreat Center Supporting HHP

Working name: [NEEDS: name]. Built from my capstone proposal, "From Study to Readiness." Every market and cost-of-inaction figure below comes from that deck and the sources it cites. Every operating figure is an assumption, marked as one, until I replace it with a quote.

1. Executive Summary

Hawaiʻi has about 104,765 veterans. Applying the national PTSD rate, roughly 7,650 of them have PTSD right now. Today, a veteran who wants breakthrough-therapy care has to leave the islands to get it. The Heroic Hearts Project (HHP) already sends veterans to psychedelic-assisted retreats outside the United States [verify how HHP describes its program]. The weak link for a Hawaiʻi veteran is everything around the retreat: preparation before they fly out, and integration after they come home, which is where outcomes are either kept or lost.

The center fills that gap in two phases.

- **Phase 1 (2027–2029): legal today, asset-light.** We run preparation and integration retreats for HHP veterans and their families in rented Hawaiʻi retreat venues, with nothing controlled on site. We also host the veteran-advocate and testimony training from my capstone. Ten veterans per cohort: 6 cohorts in Year 1, 10 in Year 2, 16 in Year 3. That is 320 veterans over three years.
- **Phase 2 (after FDA approval and state authorization): treatment site.** The same program, staff and relationships become a site that delivers FDA-approved MDMA- or psilocybin-assisted therapy in Hawaiʻi, paid for by VA Community Care, TRICARE and insurers. My capstone's Ask A, a JABSOM-led readiness task force, is the policy path that makes Phase 2 possible.

Funding need: about $261,000 in Year 1, $513,000 in Year 2 and $784,000 in Year 3, each including a 10% reserve. The fully loaded cost is $4,000–$4,700 per veteran. The organization is a nonprofit and veterans attend at no cost, so the money comes from grants, donors, sponsors and an HHP partnership, not from fees.

Why it is worth funding: the capstone deck estimates PTSD's excess cost among Hawaiʻi's veterans at about $196 million a year, or $25,684 per veteran with PTSD, in 2018 dollars (Davis et al. 2022). One fully loaded seat costs less than a fifth of one year of that excess. That comparison is the reason the problem is worth funding. It is not a promise of savings: integration support is not the treatment, and Phase 1 will not claim the outcomes the treatment trials report.

2. The Problem and the Opportunity

Need. The figures come from the capstone deck.

- 6,398 U.S. veterans died by suicide in 2023, about 17.5 a day. Hawaiʻi lost 20 in 2022 (VA 2025 report; Hawaiʻi data sheet 2022).
- Veterans with PTSD are 2.4× likelier to be unemployed and 3.8× likelier to be disabled (Pietrzak et al. 2023).
- PTSD care has seen 20 years of little innovation, and SSRIs and talk therapy often fail.

- Two compounds, MDMA and psilocybin, hold FDA Breakthrough Therapy designation. In the MAPP2 trial, 71% of MDMA-assisted therapy participants no longer had PTSD, against 48% on placebo plus therapy (Nature Medicine 2023).

Demand is already proven, and the money is leaving. Veterans pay $4,000–$8,000 for a five-day stay in Tijuana, before airfare from Hawai‘i (KPBS 2024). Oregon’s licensed psilocybin program served 5,935 clients in 2025, which scales to about 2,000 a year at Hawai‘i’s population (Oregon Health Authority 2025; order-of-magnitude only). Hawai‘i patients spent more than $230M on out-of-state cancer care in 2021–23 (UH Cancer Center 2026), which shows the same pattern: when care isn’t local, the dollars go too.

The gap the center fills. A veteran from Hilo or Wai‘anae who goes to an HHP partner retreat today prepares alone and comes home to no local peer group. That is a gap in the care itself, and a local organization can close it legally now.

Market size (Phase 1). About 7,650 Hawai‘i veterans have PTSD; that floor excludes active duty, first responders and families. Phase 1 serves 60–160 veterans a year: under 1% of that number in Year 1 and about 2% by Year 3. Capacity is limited by funding, not demand. [NEEDS: how many Hawai‘i veterans HHP serves a year, and its waitlist. This is the number that proves the Phase 1 market and is worth getting from HHP before anything else.]

3. The Organization

- **Mission:** [NEEDS: my wording]. Working version: bring the care around breakthrough therapies home to Hawai‘i’s veterans now, and be ready to deliver the therapies here the day they are approved.
- **Legal form:** a Hawai‘i nonprofit, operating under a fiscal sponsor in Year 1 so it can take grants before IRS determination. It files its own Form 1023 in Year 1 and incorporates with DCCA. [NEEDS: fiscal sponsor. Ask HHP whether it sponsors projects, then a Hawai‘i community foundation.]
- **Relationship to HHP:** a program partner under an MOU, not a subsidiary. HHP refers and screens veterans and runs the retreats abroad. The center runs preparation, integration and family programs in Hawai‘i. The MOU covers referrals, data sharing with consent, use of HHP’s name, and who carries clinical responsibility at each stage. [NEEDS: confirm HHP’s interest and terms. Everything in this plan depends on it.]
- **Relationship to Healing Breakthrough:** the center hosts Healing Breakthrough’s grassroots training for Hawai‘i advocates and follows its framing guidance: “breakthrough therapies,” no decriminalization language, no ceremony imagery.

4. Programs and Services (Phase 1)

Program	Format	Who	What it delivers
Preparation series	4 virtual sessions in Year 1; an in-person 2–3 day retreat from Year 2 if funded	Veterans accepted by HHP, before departure	Intentions, expectations, medical and safety briefing, peer cohort formed before travel
Integration retreat	5 days / 4 nights, cohort of 10	Veterans 2–8 weeks after their HHP retreat	Peer-led integration circles, a licensed clinical lead, ocean and ‘āina-based activity, sleep, nutrition, a plan for the next 6 months

Program	Format	Who	What it delivers
Follow-up circles	6 monthly virtual sessions	Every graduate	Keeps the cohort together; early warning if someone is struggling
Family program	1–2 days per integration cohort [Year 2]	Spouses and partners	What changed, what to expect, how to support
Advocate training	1-day sessions during session season	Graduates who choose to advocate	The 3-minute story, testimony writing and delivery: the capstone's Deliverable 2

Scope boundaries, written down so they hold: - No controlled substances on site, and no referral to illegal use in Hawai‘i. Phase 1 serves veterans going through HHP's legal-abroad pathway only. - Clinical work is done only by Hawai‘i-licensed clinicians (psychologist, LCSW or LMHC) acting within their license. Integration coaching is described as peer support, not therapy. - Every cohort has a written crisis and suicide-risk protocol: screening at intake, a clinician on call, the Veterans Crisis Line (988, press 1), and a warm handoff to VA care. - Stories are shared at each person's comfort level, nothing is filmed without written consent, and reports are anonymized. These rules carry over from the HB training (deck slide 18).

Culture and place. Native Hawaiian practitioners are partners, paid and credited, and ‘āina-based activities happen only with their guidance and permission. The deck names Native Hawaiian and cultural leaders as part of the coalition for culturally informed care. This is about partnership, not program imagery, which keeps it consistent with the messaging rules. [NEEDS: named partners.]

5. Marketing and Outreach

Veterans attend free, so "marketing" means two things: getting referrals and raising money.

- **Referrals:** HHP intake first. Then the Hawai‘i Office of Veterans' Services, VFW and American Legion posts, Vet Centers, unit and alumni networks, and word of mouth from graduates, which is the channel that matters most.
- **Funders:** veteran-focused foundations, Hawai‘i family and community foundations, and corporate sponsors with a military presence in Hawai‘i. Individual donors are reached through one annual event and graduates' stories, shared with consent.
- **Message:** the deck's "Say/Avoid" list applies. Lead with outcomes and the cost of inaction; never debate.

6. Operations

- **Facilities:** rent existing licensed retreat venues per cohort, rotating islands so neighbor-island veterans aren't always the ones who fly. No property in Phase 1: leasing waits for steady funding and Phase 2 licensing requirements. [NEEDS: three venue quotes, per person per night, all-inclusive.]
- **Staffing:**
  - Year 1: a part-time program coordinator (0.5 FTE), contracted clinicians and peer facilitators per cohort, and me as unpaid founder and director alongside my day job.
  - Year 2: a half-time paid executive director.
  - Year 3: a full-time executive director.

  - Facilitators are HHP alumni trained in peer integration.
- **Board:** 5–7 seats: a veteran, a clinician, a Native Hawaiian community leader, a finance and nonprofit-governance member, an HHP liaison, and a JABSOM or academic member once Phase 2 planning begins.
- **Measurement:**
  - Validated measures at intake, discharge, 3 months and 6 months: PCL-5 for PTSD symptoms and PHQ-9 for depression [confirm with clinical lead].
  - Follow-up attendance, graduates who become advocates, and testimonies submitted.
  - Results reported to funders in aggregate only.

7. Leadership

I am a retired U.S. Marine Corps Lieutenant Colonel. I commanded 3rd Reconnaissance Battalion and served as the Marine Forces Central Command Inspector General, and I am now a Senior Project Manager at NeoGenomics Laboratories and a University of Hawai‘i DLEMBA student. Command gave me the leadership and accountability. Inspector General work gave me the habit of building compliance in from the start, which this venture needs more than most. Project management at a clinical laboratory gave me the operational discipline. [NEEDS: my HHP connection, in my own words. The deck's opening slide calls it personal, and it belongs here.]

8. Financial Plan (Phase 1)

Assumptions (every line is a placeholder until quoted)

Item	Assumption	Basis
Cohort size	10 veterans, 5 days / 4 nights	Program design
Venue, lodging, meals	$250 per person per night	[NEEDS: venue quotes]
Clinical lead	$1,000 per day	[NEEDS: clinician rate]
Peer facilitators	2 at $500 per day	[NEEDS: rate]
Inter-island travel	$250 per veteran	[NEEDS: fares]
Materials, ground transport, event insurance	$1,500 per cohort	Estimate
Follow-up circles	$300 per veteran	6 sessions, facilitator time
Program coordinator	$70,000 × 1.3 loaded; 0.5 FTE Y1, 1.0 FTE Y2–3	[NEEDS: Hawai‘i salary data]
Executive director	$110,000 × 1.3 loaded; 0 Y1, 0.5 FTE Y2, 1.0 FTE Y3	[NEEDS: comparable roles]
Liability and professional insurance	$8k / $10k / $14k	[NEEDS: broker quote]
Legal, accounting, formation, audit	$12k / $8k / $10k	Includes 1023 filing in Y1
Outreach and fundraising	$6k / $10k / $15k	Estimate
Technology and admin	$4k / $6k / $8k	Estimate
Reserve	10% of total cost	Policy

Cost per cohort

Line	Amount
Venue, lodging, meals (10 × 4 nights × $250)	$10,000

Line	Amount
Staff ((1,000 + 2 × 500) × 5 days)	$10,000
Inter-island travel	$2,500
Materials, transport, event insurance	$1,500
Follow-up circles (10 × $300)	$3,000
Direct cost per cohort	**$27,000 ($2,700 per veteran)**

Three-year budget

	Year 1	Year 2	Year 3
Cohorts / veterans	6 / 60	10 / 100	16 / 160
Direct program cost	$162,000	$270,000	$432,000
Program coordinator	$45,500	$91,000	$91,000
Executive director	$0	$71,500	$143,000
Insurance	$8,000	$10,000	$14,000
Legal and accounting	$12,000	$8,000	$10,000
Outreach and fundraising	$6,000	$10,000	$15,000
Technology and admin	$4,000	$6,000	$8,000
Total operating cost	**$237,500**	**$466,500**	**$713,000**
Reserve (10%)	$23,750	$46,650	$71,300
Funding need	**$261,250**	**$513,150**	**$784,300**
Fully loaded cost per veteran	$3,958	$4,665	$4,456

Not in this budget: the in-person preparation retreat, the family program, and the virtual preparation sessions' facilitator time. Each needs its own cost line before it runs. [NEEDS: cost these.] Advocate training is assumed to be delivered with Healing Breakthrough at no cost to the center.

Cost per veteran rises in Year 2 because the director's salary arrives before the cohort count catches up. Year 3 gets it back down.

Funding mix (targets, not commitments)

Source	Share	Year 1	Year 2	Year 3
Foundation grants	40%	$104,500	$205,260	$313,720
Individual donors and annual event	25%	$65,313	$128,288	$196,075
Corporate and community sponsors	15%	$39,188	$76,973	$117,645
HHP partnership (co-funded seats or grant)	10%	$26,125	$51,315	$78,430
Earned (advocate-training hosting, venue partners, sliding-scale family program)	10%	$26,125	$51,315	$78,430

[NEEDS: at least one anchor grant or pledge before launching Year 1. The plan does not start the first cohort until two cohorts of direct cost, $54,000, are in hand.]

Sensitivity

- **Venue cost** is the largest uncertain line. At $400 a night instead of $250, each cohort costs $6,000 more: about $36,000 more in Year 1 and $96,000 more in Year 3.
- **Missed fundraising targets:** the plan scales by cohort, not by headcount. Each cohort not run saves $27,000, and in Year 1 fixed overhead is only $75,500. That is why the director's salary waits until Year 2.

9. Phase 2: Treatment Site

Gates. Phase 2 does not start until all three of these are true: 1. The FDA has approved MDMA- or psilocybin-assisted therapy, and the drug is rescheduled. [NEEDS: current FDA and DEA status at the time of writing.] 2. Hawai'i has the readiness framework (task force recommendations, provider rules) that my capstone's Ask A is working toward. 3. Payers, meaning VA Community Care, TRICARE and HMSA or other insurers, will reimburse the course.

What changes: a leased dedicated site, DEA registration, whatever certification the FDA's approval requires of treatment sites, and a full clinical staff. The Phase 1 cohort model, integration program and referral network carry straight over. That is the reason to build them first.

Size of the prize. The capstone deck's uptake scenarios put care spending at $2.3M, $4.6M and $9.2M a year for 380, 765 and 1,530 veterans, priced at the $6,000 Tijuana midpoint. A U.S. FDA-approved course will likely cost more, so the dollar figures are a floor and the veteran counts are the real constraint. These are scenarios, not forecasts. Phase 2 gets its own financial model once the gates are visible.

Research bridge. Before approval, the site can partner with JABSOM to host clinical trials, which the deck identifies as a federal funding opportunity. This is the earliest legal way to deliver the therapy itself in Hawai'i.

10. Risks and Mitigations

Risk	Mitigation
Seen as facilitating illegal drug use	Nothing controlled on site; HHP's legal-abroad pathway only; written scope; legal review before launch
A participant in crisis	Intake screening, clinician on call, written suicide-risk protocol, 988 press 1, VA handoff
HHP partnership doesn't happen or changes	Get the MOU before fundraising; integration also serves veterans from clinical trials and other legal pathways
Fundraising falls short	Cohort-by-cohort scaling, two-cohort cash rule, low fixed cost in Year 1
Clinical scope creep (coaching that becomes therapy)	Licensed clinical lead, written role boundaries, insurance matched to scope
Cultural appropriation or backlash	Paid Native Hawaiian partners on the board and in programming; no ceremony imagery
Founder capacity (I have a day job)	Coordinator from Day 1, paid director in Year 2, board with real responsibilities

Risk	Mitigation
Phase 2 never arrives	Phase 1 stands on its own: it supports veterans whatever happens to the law

11. Milestones

When	Milestone
Nov–Dec 2026	Talks with HHP; MOU drafted; fiscal sponsor chosen; venue and clinician quotes replace the assumptions above
Jan–Mar 2027	Board seated; anchor grant applications in; Form 1023 filed; first advocate training runs alongside the 2027 session (capstone)
Apr–Jun 2027	Two-cohort cash threshold reached; protocols and legal review complete
Jul–Dec 2027	First 6 integration cohorts; outcome measures collected
2028	10 cohorts; family program; half-time executive director; Year 1 outcome report to funders
2029	16 cohorts; Phase 2 feasibility study with JABSOM if the gates are moving

12. Open Items Before This Plan Is Real

1. HHP's interest, the terms of an MOU, and its Hawai‘i referral numbers.
2. Venue, clinician, insurance and salary quotes to replace every assumption in Section 8.
3. A fiscal sponsor, and one anchor funder.
4. Legal review of the Phase 1 scope.
5. Named Native Hawaiian partners.
6. The mission statement and leadership story, in my words.

-Drafted with help from Claude (Anthropic, 2026); reviewed and edited by me.